

Psych Consult - Lethargy

REASON FOR CONSULTATION:

Lethargy.

HISTORY OF PRESENT ILLNESS:

The patient is a 62-year-old white female with a past medical history of left frontal glioblastoma with subsequent craniotomy infection for PE

DVT

hyperlipidemia

and hypertension who is according to the patient's daughter expressing signs of depression. Symptoms began on February 5

2007

upon receiving the unexpected news

the patient would need three to four more days of chemotherapy and radiation therapy for her glioblastoma

described as a sudden onset of symptoms including hypersomnia (18 to 20 hours per day)

drastic decrease in energy level

anhedonia

feelings of hopelessness and helplessness

psychomotor retardation

and past history of suicidal ideations. The patient's appetite is unknown since she had been fed by NG tube after being diagnosed with neuromuscular oropharyngeal dysphagia. Prior to receiving the news for needing more cancer therapy

the patient was described as being "fine

" participating in physical therapy and talking regularly as she was looking forward to leaving the hospital. Now

the patient has become angry

socially withdrawn

not wanting to see anyone including her own grandchildren

and not participating in physical therapy. Has been on a daily dose of Lexapro since January 08

2007

was increased from 10 mg to 20 mg on January 24

2007

which is her current dose. Has been on Provigil 100 mg b.i.d. since February 06

2007

but has not noticed an impact. Had been on Zyprexa 2.5 mg p.o. q.p.m. from December 20

2006

to February 01

2007

but has been discontinued. Currently
the patient has not displayed any manic symptoms
auditory or visual hallucinations
or symptoms of anxiety. Also
denies any homicidal ideations.

PAST PSYCHIATRIC HISTORY:

Was prescribed Prozac for depression
felt during husband's successful battle with prostate cancer. Never been diagnosed with psychiatric illness.
Displayed some psychotic symptoms
status post craniotomy while in ICU
treated with Zyprexa and Xanax during hospitalization in 2006.

PAST MEDICAL HISTORY:

Craniotomy November 2006 with subsequent CSF infection of enterobacter
status post glioblastoma multiforme
PE
DVT
hypertension
SIADH
and IVC filter. No history of thyroid problems
seizures
strokes
or traumatic head injuries.

HOME MEDICATIONS:

Norvasc 5 mg daily
TriCor 145 mg daily
aspirin one tablet daily
Tylenol
and glucosamine chondroitin sulfate.

CURRENT MEDICATIONS:

Norvasc 10 mg p.o. daily

Decadron injection 6 mg IV q.12h.

Colace 100 mg liquid b.i.d.

Cardura 2 mg p.o. daily

Lexapro 20 mg p.o. daily

Lopressor 50 mg p.o. q.12h.

Flagyl 500 mg via PEG tube q.8h.

modafinil 100 mg p.o. b.i.d.

Lovenox 60 mg subcu q.12h.

insulin sliding scale

Tylenol suppositories 650 mg rectal q.4h. p.r.n.

and Ambien 5 mg p.o. q.h.s. p.r.n.

ALLERGIES:

PHENYTOIN (STEVENS-JOHNSON SYNDROME)

CODEINE

NOVOCAIN

UNKNOWN ALLERGY.

FAMILY MEDICAL HISTORY:

Father had lung cancer

was smoker for 40 years. Father's aunt have heart disease.

SOCIAL AND DEVELOPMENTAL HISTORY:

Currently lives with husband of 40 years in League City

has a Masters in Education

is a retired reading specialist which she did it for 33 years. Has one younger brother

one daughter. Denies use of tobacco

alcohol and illicit drugs. The child as per daughter was picked on and has a strained relationship with her mother

but they still are communicating.

MENTAL STATUS EXAMINATION:

The patient is a 62-year-old white female

lying in hospital bed

with gown on

eyes closed

short shaven hair

and golf ball-sized indentation in the anterior fontanelle from craniotomy. Psychomotor retardation
poor eye contact
speech low volume
slow rate
poor flexion
essentially unresponsive
and somnolent during interview. Poor concentration
mood unknown (the patient did not respond to questions)
affect flat
thought process logical and goal directed
thought content unable to assess from the patient but the patient's daughter denied delusions and
homicidal ideations. Positive for passive suicidal ideations and perceptions. No auditory or visual
hallucinations. Sensorium stuporous
did not answer orientation questions. Memory information
intelligence
judgment
and insight unknown.
Mini-Mental status examination unable to be performed.

ASSESSMENT:

A 62-year-old white female status post craniotomy for glioblastoma multiforme with subsequent CNS
infection and currently has been displaying symptoms of depression for the past seven days and hence
was told she needed more chemotherapy and radiation therapy.

Axis I: Depression
NOS. Rule out depression secondary to general medical condition.

Axis II: Deferred.

Axis III: Craniotomy with subsequent CSF infection

PE
DVT
and hypertension.

Axis IV: Hospitalization.

Axis V: 11.

PLAN:

Continue Lexapro 20 mg p.o. daily. Discontinue Provigil
begin Ritalin 5 mg p.o. q.a.m. and q. noon.

Thank you for the consultation.